NAME	VEDANT PATOLE 20Y/M	**REGISTRATION DATE TIME**	-
AGE/GENDER	020Y / MALE	**STUDY DATE TIME**	11-JUL-2026 16:25
REGISTRATION ID	-	**REPORT DATE TIME**	12-JUL-2026 00:47
REF. BY	-	**MODALITY**	MR
PATIENT ID	29932	**ACCN NO.**	-

MRI RIGHT SHOULDER JOINT

Technique: Multiplanar, multisequence MRI of the right shoulder was performed without intravenous contrast.

Clinical History: Shoulder pain.

Prior Imaging: No prior imaging available.

Findings:

Bones:

- **There is a well-circumscribed intramedullary lesion centred within the proximal humerus, involving the humeral head, anatomic/surgical neck region, and proximal metaphysis, measuring approximately 4.6 x 3.7 x 4.0 cm.**
- **The lesion demonstrates a multiloculated internal architecture with a few thin septations.**
- **No surrounding marrow oedema is identified.** No aggressive periosteal reaction, cortical breakthrough, or extraosseous soft tissue component is evident on the provided examination.
- **No associated pathological fracture is seen.** The remaining visualised osseous structures of the shoulder girdle demonstrate preserved marrow signal. No Hill-Sachs lesion or glenoid bone loss.

Rotator Cuff Complex:

- **Supraspinatus:** Tendon is intact with preserved thickness and signal. No partial-thickness or full-thickness tear. No significant tendinosis.
- **Infraspinatus:** Intact without tendinosis or tear.
- **Subscapularis:** Intact without tear or medial subluxation of the biceps pulley complex.
- **Teres Minor:** Intact.
- No rotator cuff muscle oedema or fatty atrophy.

Biceps Tendon:

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- Long head of the biceps tendon is normally positioned within the bicipital groove and is intact throughout the visualised course. No tenosynovitis, split tear, or subluxation/dislocation.

Labrum:

- No discrete labral tear is identified on this non-arthrographic examination. No paralabral cyst.

Glenohumeral Joint:

- No glenohumeral joint effusion, synovial thickening, or intra-articular loose body.
- Articular cartilage is preserved without focal full-thickness chondral defect identified on this study.
- Axillary recess and rotator interval are unremarkable.

Acromioclavicular (AC) Joint:

- No significant degenerative arthropathy, marrow oedema, capsular hypertrophy, or joint effusion at the AC joint.
- No significant mass effect upon the supraspinatus outlet.

Acromion / Coracoacromial Arch:

- No subacromial spur or morphologic narrowing of the subacromial space is evident on this examination.

Bursa:

- No subacromial-subdeltoid bursitis. No fluid distension of the subcoracoid bursa.

Soft Tissues / Musculature:

- Deltoid and visualised periscapular musculature are preserved without oedema, strain pattern, or fatty atrophy.
- **No adjacent soft tissue mass is identified in relation to the proximal humeral lesion.**

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CHAAR SAHIBZADE MRI AND CT SCAN CENTRE

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Impression:

1. **Well-defined non-aggressive appearing intramedullary lesion within the proximal humerus involving the humeral head, neck, and proximal metaphysis, measuring up to 4.6 cm, with thin internal septations and no surrounding marrow oedema, cortical destruction, soft tissue extension, or pathological fracture.**
2. Imaging features favour a benign primary osseous lesion; differential considerations include a cystic/chondroid lesion such as unicameral bone cyst/ aneurysmal bone cyst
3. No rotator cuff tear, labral abnormality, or other internal derangement of the right shoulder identified on this examination.

Recommendations:

- Orthopaedic evaluation is recommended, with consideration of referral to an orthopaedic oncologic or musculoskeletal tumour service if clinically indicated.
- If prior imaging becomes available, comparison would be helpful to establish long-term stability.
- Follow-up imaging interval may be determined based on radiographic appearance, symptoms, and specialist assessment.

Dr Amit Choure
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MBBS MD (Radiodiagnosis)
Regn No: 2014/08/3431

Dr Amit Choure
12th Jul 2026